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TO: Pam Health at Home-TX FROM: Joukovski, Olga

FAX: 512-295-7070 PAGES: 8

PHONE: 512-295-7000 DATE: 03/02/26

RE: CC:

☐ Urgent ☐ For Review ☐ Please Comment ☐ Please Reply ☐ Please Recycle

Comments:

Order No:485-25044

HOME HEALTH CERTIFICATION AND PLAN OF CARE

1. Patient's HI Claim No.	2. Start Of Care Date	3. Certification Period	4. Medical Record No.	5. Provider No.
8RN2NH2PU14	01/29/2026	01/29/2026 - 03/29/2026	9344	747688

6. Patient's Name and Address
Mccauley, Sandra (9344)
4019 DEEP RIVER
San Antonio, TX 78253
Phone: (210) 452-3262

7. Provider's Name, Address and Telephone Number
PAM Health at Home
825 Main St Ste 110
Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

8. Date of Birth: 05/14/1941 **9. Sex:** ☐ M ☑ F

10. Medications: Dose/Frequency/Route (N)ew (C)hanged

Medication	Dose	Frequency	Route	Status
Eliquis 5 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	5mg	daily	oral	LS
Metoprolol succinate ER 50 mg tablet,extended release 24 hr [Amount Of/ Dosage Form: 1 Tablet]	50mg	Daily	oral	LS
Omeprazole 20 mg capsule,delayed release [Amount Of/ Dosage Form: 1 Capsule]	20mg	Daily	oral	LS

11. ICD-9 / 10 CM **Principal Diagnosis**

Code	Code Description	Date	O/E
Z48.3	Aftercare following surgery for neoplasm		

12. ICD-9 / 10 CM **Surgical Procedure**

Code	Code Description	Date	O/E

13. ICD-9 / 10 CM **Other Pertinent Diagnosis**

Code	Code Description	Date	O/E
C23	Malignant neoplasm of gallbladder		
E11.9	Type 2 diabetes mellitus without complications		
I10	Essential (primary) hypertension		
R32	Unspecified urinary incontinence		
Z79.01	Long term (current) use of anticoagulants		

14. DME and Supplies:

DME: Bed Side Commode, Grab Bars, Tub/Shower Bench, Walker
PPE MASK, PROBE COVERS, GLOVES, diabetic supplies
Supplies: Sharps Container 1 Qrt #4829

15. Safety Measures:

911 Emergency / Protocol, Assistance with IADLs, DIABETIC PRECAUTIONS, Emergency Plan / Measures, Emergency Plan Developed, Equipment Safety, Fall / Safety Precautions, Fall Precautions, Infection Control Standard Measures and Precautions, Keep Pathways Clear, Keep Pathways Clear and Safe, Medication Safety / Storage, Safe Disposal of Sharp, Contaminated

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Materials, Safety in ADLs, Slow Position Change, Standard Precautions/Infection Control, Support During Transfer and Ambulation, Universal Precautions, Use of Assistive Devices, Walker / Cane

16. Nutritional Req.
Protein Hi Diet, No Concentrated Sweets, No Added Salt,
Heart healthy

17. Allergies:
Medication Allergies: niacin

18.A. Functional Limitations
☐ 1- Amputation ☐ 4- Hearing ☐ 7- Ambulation
☑ 2- Bowel/Bladder (Incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☑ 6- Endurance ☐ 9 - Legally Blind
☐ A- Dyspnea with minimal exertion
☑ B- Other (Specify): Abdominal binder required when OOB

18.B. Activities Permitted
☐ 1- Complete Bedrest ☑ 5- Exercises Prescribed ☐ 9- Cane
☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☐ 3- Up as Tolerated ☐ 7- Independent at Home ☑ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☑ D- Other (Specify):Abdominal binder when OOB SBA

19. Mental Status
☑ 1 - Oriented ☐ 2 - Comatose ☐ 3 - Forgetful ☐ 4 - Depressed ☐ 5 - Disoriented ☐ 6 - Lethargic ☐ 7 - Agitated
☐ 8 - Other(Specify):

20. Prognosis: ☐ 1- Poor ☐ 2- Guarded ☐ 3- Fair ☑ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify):

21. Treatments and Orders for Disciplines:
Frequency
PT Frequency: 1W5 Starting Week of 01/29/2026,DC OT ORDER,DC ST ORDER,SN Frequency: 1W3, 1EOW6,SN PRN Visits/Hours: 3 (Surgical complications)

Initial Order received:
Yes

Vitals Normal Range
BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100
Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

Vitals Physician Alert Range
Temperature: < 96.5 or > 100.5
Pulse: < 56 or > 100
Respiratory: < 12 or > 28

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BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Fasting Blood Sugar: < 70 or > 250
Random Blood Sugar: < 70 or > 300
Weight: < 171 or > 181
Pain: > 7

SN

BEST PRACTICE FALL PREVENTION:

- Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION:

- Clinician to perform drug regimen review/assessment to assess for potential clinically significant medications issues.
- Clinician to report potentially clinically significant medications issues to physician within 1 calendar day.
- Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

BEST PRACTICE PRESSURE ULCER PREVENTION:

- Clinician will perform validated test BRADEN to assess patient for risk of development of Pressure ulcers.
- Clinician will perform teaching and training to patient/caregiver for pressure ulcer prevention.

GENERAL: BEST PRACTICE DIABETES:

- Clinician to assess/perform monitoring for the presence of skin lesions on the lower extremities and perform patient/caregiver education on diabetic foot care.

GENITOURINARY:

- Clinician to obtain Urine for U/A and C& S as needed for s/s of UTI via clean catch and notify physician of results.
- Clinician to assess patient for presence of bowel and bladder incontinence. Instruct patient /caregiver measures to improve incontinence.

Instruct patient/caregiver on signs/symptoms of UTI to report to SN/MD.

SURGICAL WOUND:

- Clinician to assess surgical wound, instruct/perform wound care as follows: Using aseptic technique monitor surgical site for s/s of infection and report any abnormalities to MD. Clinician to instruct patient / caregiver on surgical wound care as follows: Monitor for s/s of infection to abdominal surgical site, report redness, drainage or fever to nurse. Wear abdominal binder when OOB. Home health to provide wound care supplies as ordered. Clinician to discontinue wound care and wound care supplies when wound is healed.

HOSPITALIZATION RISK FACTORS:

- Clinician will instruct the patient and/or caregivers on the following unstable disease processes that may lead to unplanned hospitalizations:HTN, DMII, GALLBLADDER CA S/P RESECTION
- Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc
- Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.

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- Clinician will assess and teach patient and/or caregivers regarding declines in mental, emotional, or behavior status and when to report to the MD.
- Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.
- Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

CARDIOVASCULAR - HTN:

- SN to instruct pt/cg on ways to manage HTN such as taking medications as prescribed, increasing physical activity, increasing foods/fluids high in potassium and calcium, managing stress, and avoiding high sodium foods, limit alcohol and monitor blood pressures at home.
- SN to instruct patient/caregiver that uncontrolled HTN can lead to heart attack, stroke, heart failure, kidney complications, vision loss, metabolic syndrome and vascular dementia. SN to instruct pt/cg that HTN is known as the silent killer and symptoms may not be obvious.
- SN to instruct the patient/caregiver on the following symptoms could be signs of a heart attack: chest discomfort, discomfort in one or both arms, back, neck, jaw, stomach, shortness of breath, cold sweat, nausea, or dizziness. Instruct patient/caregiver on signs and symptoms that necessitate calling 911.
- SN to instruct patient/caregiver on Heart Healthy diet.

PT

PHYSICAL THERAPY:

- Evaluation and Treatment
- Establish Home Exercise Program and Activity Program
- Home safety evaluation and fall prevention/teach fall risk reduction strategies
- Therapeutic Exercises: Balance, coordination, and/or agility training
- Therapeutic Exercises: Strength training
- Gait Training: Even and uneven surfaces and/or natural terrain
- Transfer Training : Supine <>Sit, Sit <> Stand, Bed <> Chair, On/Off toilet or commode, In/Out shower or tub , Floor , car
- Bed Mobility: Scoot/roll/bridge

Assessment Summary

FOCUS OF CARE:s/p gallbladder cancer resection
CLINICAL SUMMARY: patient is an 84-year-old female recently hospitalized for gallbladder cancer resection patient lives alone and a single-story home with two step entry and one handrail daughter lives seven driveways down and is willing and able to assist patient with ADL/IADL'S as needed. Her diagnosis include DMII, urinary incontinence and hypertension. Patient and caregiver are in agreement to HH services and plan of care consents were obtained and headed to toe assessment performed midline abdominal incision noted with 40 intact staples and no signs and symptoms of infection noted. Patient is to wear abdominal binder went out of bed. She currently uses a walker for ambulation and states that her goal is to move around in her own house on her own. Patient's last PCP visit was on. 1/28/26. Daughter was unable to stay for entire. SNV BUT IS VERY ACTIVE AND HER MOTHER'S CARE. PATIENT LEFT IN CARE OF SELF FOLLOWING SNV AND STABLE CONDITION AND POSITIVE MOOD. SN REPORTED START OF CARE VIA VOICEMAIL AT. 2:16pm to Doctor Olga Joukovski's office.

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).

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All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify that the patient is confined to his/her home and needs intermittent skilled nursing, physical therapy, occupational therapy and/or speech therapy for the next 60 days.
I attest to the following:
1. Sandra McCauley is under my care.
2. I have received the following patient records from PAM Health at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
e. These patient records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.
I certify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy for the next 60 days. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter has occurred BY KRISTINA S WINTERS, NP AND ZUNAIR MUSTAQ, MD on 01.24.2026 and it is related to the primary reason the patient requires home health services. PAM HEALTH AT HOME, HOME HEALTH AGENCY, MAY PERFORM SOC ON 1/29/26.

Risk of Hospitalization

Risks: Multiple hospitalizations (2 or more) in the past 6 months; Multiple emergency department visits (2 or more) in the past 6 months; Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently reports exhaustion;
Interventions: Referrals - SN,PT,OT; Medication Management - Reconciliation, Assess patients: Knowledge,Ability,Resources & Adherence, Education; Patient/Family Education; Phone Monitoring; Front-loading Visits; Fall Prevention Program; Individualized Patient Emergency Care Plan; Care Coordination (Physicians, Hospitals, Nursing homes...); Tele monitoring
Risk Factors: Discharged from Hospital or Skilled Nursing Facility (M1000); Help with managing Medications needed (M2020); Lives Alone (M1100) ; Inadequate Support Network (M1100); Dyspnea (M1400) ; ADL Assistance Needed (M2102 & M2110)

Psychosocial Assessment

No problem found

Cognitive Status

Cognitive status: No behaviors demonstrated

Advance Directive

POA
Patient's Advance Directive - Durable Power of Attorney for Health Care (Name: Dear rice Driffill, Address: San Antonio Tx (7

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houses away), Phone: (210) 452-3262, Relation: DTR);
Have an Advanced Care Plan and it is documented in the medical record: Yes - Medical Treatment Preferences;
Have a Surrogate Decision Maker and information documented in the medical record: Yes;

Homebound Status

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Need assistance for all activities; Residual weakness; Require assistance to ambulate; Unable to safely leave home unassisted; Dependent upon adaptive device(s);
- Patient Chronic and Stable: Yes
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments: Pt is homebound due to having Multiple medical problems and functional impairment, the patient requires use of assistive
devices and the assistance of another person in order to leave home. It requires considerable effort that is very taxing on the patient to leave home.

22. Rehabilitation Potential/Discharge Plans/Goals :

Rehab Potential

Good to achieve stated goals with skilled interventions and patient's compliance with plan of care.

Discharge Plan

Discharge when medical condition is stable and patient is no longer in need of skilled services

Goals

SN

BEST PRACTICE FALL PREVENTION: - LTG
- LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION: - LTG
- LTG-Patient /caregiver will be able to name s/s of adverse side effects to report to MD by end of episode.
- LTG-Patient /caregiver will be able to verbalize/demonstrate correct dose, route, and frequency of all medications and will be able to recall adverse effects to report to MD by end of episode.

BEST PRACTICE PRESSURE ULCER PREVENTION: - LTG
- LTG-Patient/Caregiver will demonstrate/verbalize understanding of methods to prevent pressure ulcers by end of episode.

GENERAL: BEST PRACTICE DIABETES: - STG
- STG-Patient/Caregiver will demonstrate ability to perform proper diabetic foot care as evidenced by Keeping a daily log by week 4.

SURGICAL WOUND: - STG
- STG-Patient/Caregiver will be able to identify 2 s/s of infection of surgical wound by week 4.

PATIENT STATED GOALS: - LTG
- LTG-****Patient stated LTG ****To move around in my house on my own.

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HOSPITALIZATION RISK FACTORS: - LTG
- Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

CARDIOVASCULAR - HTN: - LTG
- Patient/caregiver will be able to verbalize ways to manage HTN by end of episode.

23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)
Verbal SOC Date: 01/29/2026
Signed By: Percival, Synina - RN on 01/29/2026 09:11 AM, Cosigned by Strait, Donna - RN

25. Date HHA Received Signed POT:

24. Physician's Name and Address
OLGA JOUKOVSKI MD
3303 Rogers Rd., Suite 120
San Antonio, TX 78251,NPI: 1437256179, License#: L3705,Phone: (210) 520-5040, Fax: (210) 520-5232

26. I certify/ recertify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter occurred (or will occur) within timeframe requirements and it is related to the primary reason the patient requires home health service

27. Attending Physician's Signature and Date Signed:
03/02/26

28. Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penality under applicable Federal laws.